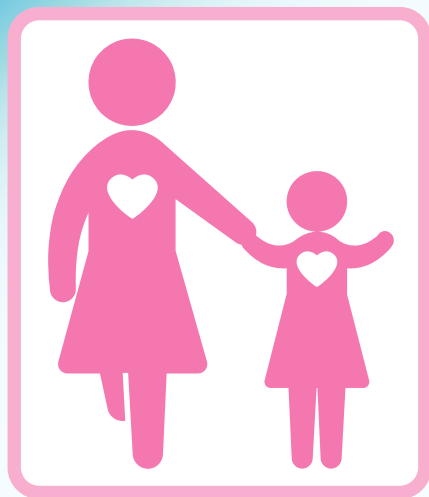




eCBHFA module

Maternal, newborn and child health (MNCH)

toolkit



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www.ifrc.org Saving lives, changing minds.

Tools per topic

topic	tools	tools	topic
Care during pregnancy	What to expect when visiting with a health worker Go to the clinic if... Healthy foods for pregnant women Birth plan checklist	After-abortion care Key messages about menstruation Facts to remember about menstrual hygiene	Post-abortion care Menstrual hygiene
Preparing for a safe birth	Baby is coming! (Birth plan checklist) Two handfuls of signs	Family planning methods Which method is best for me Using male condoms Using female condoms	Family planning
Breastfeeding	Benefits of breastfeeding How to breastfeed Common problems with breastfeeding	Reasons why female cutting still exists What religion says about female cutting Effects of female cutting on women's health	Female genital cutting
Newborn and new mother care	Kangaroo mother care Ways to keep newborn safe after birth Signs that mother is having a problem after birth (Two handfuls of signs)		

What to expect when visiting with a health worker

Care during pregnancy | A4 checklist | 1/2 - front

When you visit the health worker

A The health care worker will ask you general questions about:

Your pregnancy, such as number of weeks pregnant, first baby, past pregnancies

Your birth plan

Breastfeeding plans

Family planning

Plans for post check-up for baby and mother



B Several blood tests will be done:

Your blood pressure will be taken

Blood will be drawn to test for your blood type

Your blood will be tested for anaemia



C The health care worker will:

Measure your weight and height

Remind you to eat healthy foods and take vitamins

Check to see if you are experiencing any problems.



D Be aware!

NO TOBACCO

NO ALCOHOL

NO MEDICINES WITHOUT ASKING YOUR HEALTH CARE WORKER



What to expect when visiting with a health worker

Care during pregnancy | A4 checklist | 2/2 - back

Disease prevention checklist

A

Testing

Be sure you are tested for diseases such as Hepatitis B, HIV, Syphilis and TB.

B

Tetanus

Make sure you are up-to-date on your shots.



C

HIV

Watch viral loads and take medications as directed.



D

Malaria

Use bed net every night and take preventive drug after week 13.



E

Dengue, chikungunya, Zika

Use insect repellent, a bed net, and use condoms. Remove mosquito breeding sites.



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Go to the clinic if...

Care during pregnancy | A3 poster | 1/1

Head



If you have:

- bad or ongoing headache
- blurred vision
- swelling in the face
- a high fever
- loss of consciousness/fainting

Hands



If you have:

- swelling of the hands
- very pale palms of hands or under the nails

Belly



If you experience:

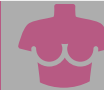
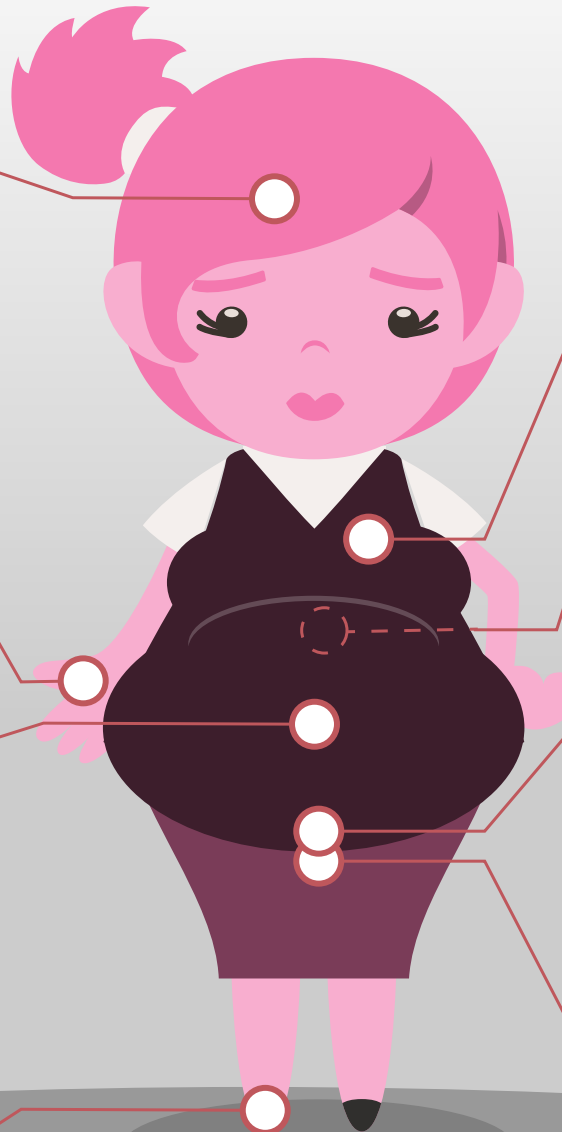
- severe pain in the belly
- severe vomiting or vomiting all the time
- intense pressure at the base of the belly
- woman stops regularly feeling the baby move inside her

Feet



If you have:

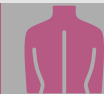
- swelling in the feet



Chest

If you have:

- difficulty breathing



Back and nerves

If you experience:

- low dull backache
- convulsions or fits (do not stop shaking)



Vagina

If you have:

- any bleeding from the vagina, especially bright red blood
- heavy vaginal discharge
- vaginal discharge that smells very bad
- sudden gush of clear, watery fluid
- ulcers or growths on the sex organs



Urinal track

If you have:

- pain when passing urine



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Healthy foods for pregnant women

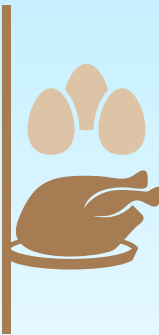
Care during pregnancy | A4 poster | 1/1

Eat more healthy food during the pregnancy to support the baby.

Drink lots of water.



Eat foods with protein and iron, such as meat and eggs. These contain iron which helps the blood stay strong.



Eat greens and fruit for more energy (e.g. spinach, cabbage, pumpkins, mango, carrots, tomatoes, avocado, oranges, pineapple, banana, potato and sweet potato)

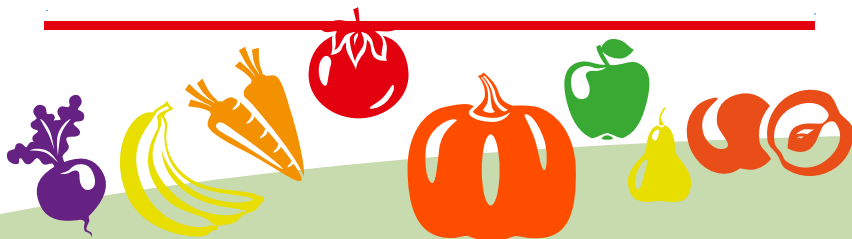


Take extra iron because a woman loses some blood after the birth.



Take the iron and folic acid tablets to help prevent anaemia.

Eat foods that help to build strong bones and fight infections, such as beans, lentils, peas, nuts, milk, yogurt, cheese, fish, eggs, chicken, meat, maize, wheat and rice.

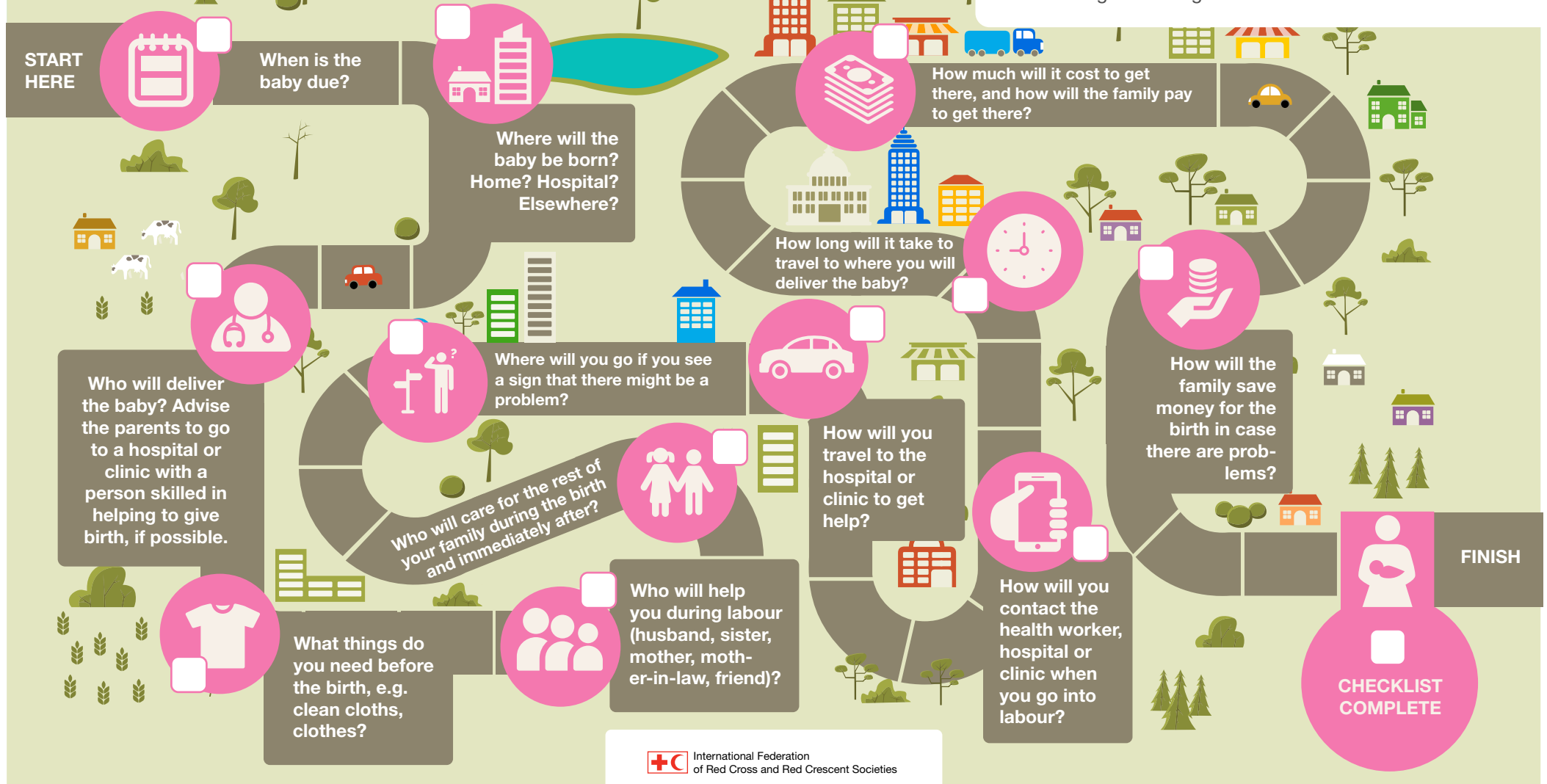


Birth plan checklist

Care during pregnancy | A4 checklist | 1/1

Make a birth plan by answering the questions on the pathway. Follow the pathway round, asking the pregnant woman and her family to think about and answer the questions. These will help her and her family be prepared for the birth, knowing that they have considered all the details.

Record the answers and revisit them on future visits to check that nothing has changed.



Baby is coming!

Preparing for a safe birth | A3 poster | 1/2

Early signs of labour



Pains, contractions (when the muscles around the belly tighten and relax) or cramps with the entire pregnant belly becoming hard for at least 30 seconds at least five times in one hour



Pressure at the base of the belly



Clear or pink-coloured liquid comes out of the vagina
Clear water comes out of the vagina
Bright red blood comes out of the vagina



Vomiting



Low, dull backache



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Baby is coming!

Preparing for a safe birth | A3 poster | 2/2

In early labour, the woman should:



Drink plenty of sweet liquids and/or tea

Take honey, if available, for energy and for soothing the throat

Eat a little if hungry



Relax her body between contractions



Take slow, deep breaths during the contractions



Pass urine often



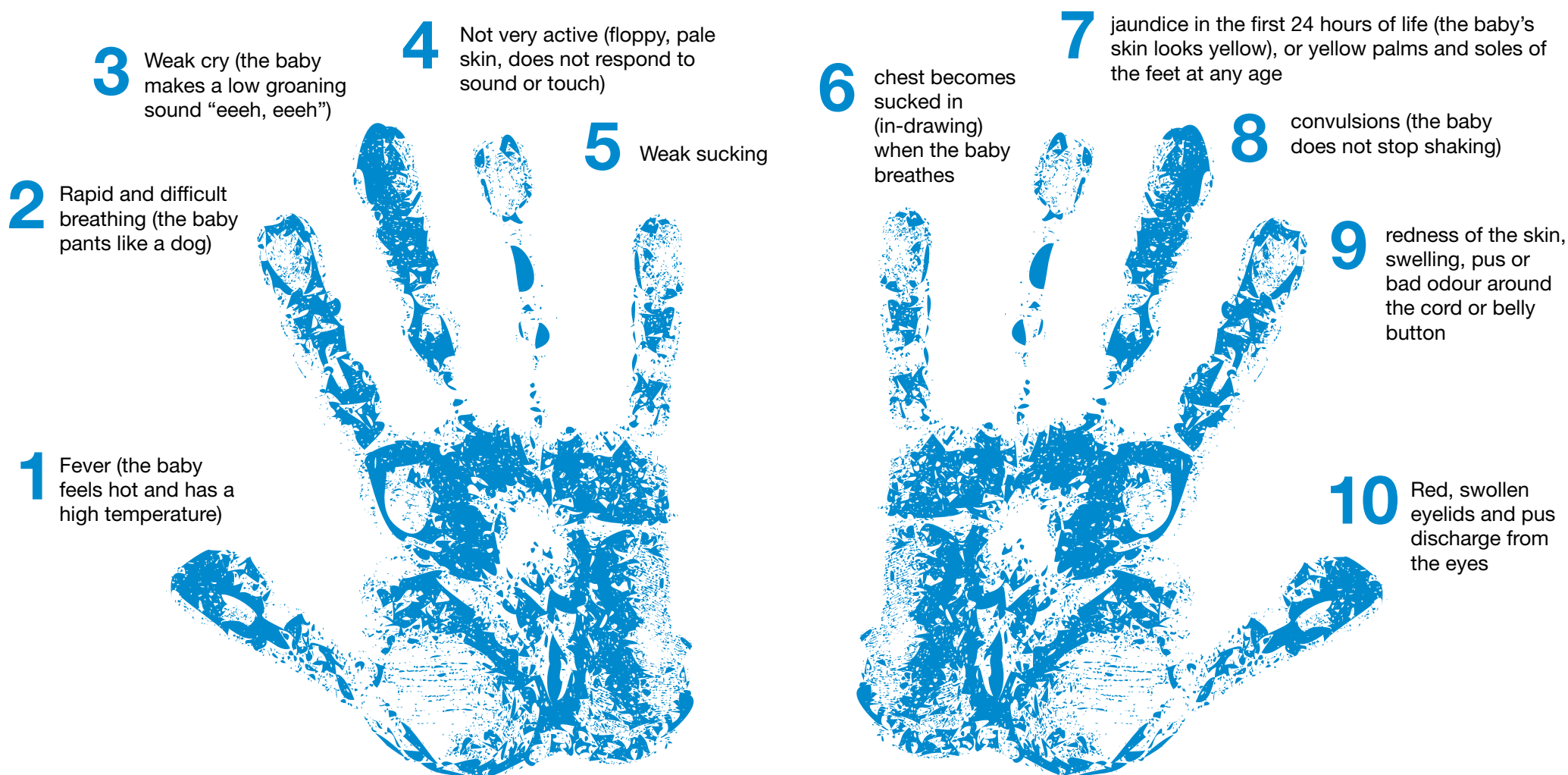
Make sure all the things needed for the birth are ready (pads, clean clothes for the mother and baby, clean and dry cloths, blankets, a hat for the newborn, soap, money for expenses)

When the contractions are five to ten minutes apart and lasting at least 30 seconds to a minute, it is time to go to the clinic or hospital or to contact the trained birth attendant, depending on the birth plan.

Two handfuls of signs

Preparing for a safe birth | Newborn and new mother care | A4 poster | 1/1

These are the signs that there may be a problem with the newborn



Benefits of breastfeeding

Breastfeeding | A4 poster | 1/1



Protects the baby from diseases if it is the only food for the first six months



Available on demand 24 hours a day



Costs nothing



Mother unlikely to get pregnant if baby is fed only breast milk for the first six months



Builds the bond between mother and baby



Helps the mother's body recover faster after the birth



Can continue up to two years or more, even when the baby starts to eat other foods



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How to breastfeed

Breastfeeding | A4 poster | 1/1

Positioning the baby



Allow baby to breathe



Removing from breast



Common problems with breastfeeding

Breastfeeding | A4 poster | 1/1

Swollen breasts ?



Try: Breastfeeding the baby often and regularly
Placing warm wet pads or cloths on the breasts after breastfeeding
Go to clinic if the breasts become hot and painful to touch, it could be an infection



Sore nipples ?

Try: Normal to feel some soreness in the first weeks of breastfeeding
Prevent soreness by making sure the baby's mouth is fully on the nipple
Put one finger gently in the baby's mouth when removing the baby from the breast
Air dry nipples dry after feeding and before covering breast
Squeeze a bit of breast milk onto finger and apply to nipple to alleviate pain
Change baby's position so baby is sucking on different place on the breast

Not enough milk ?

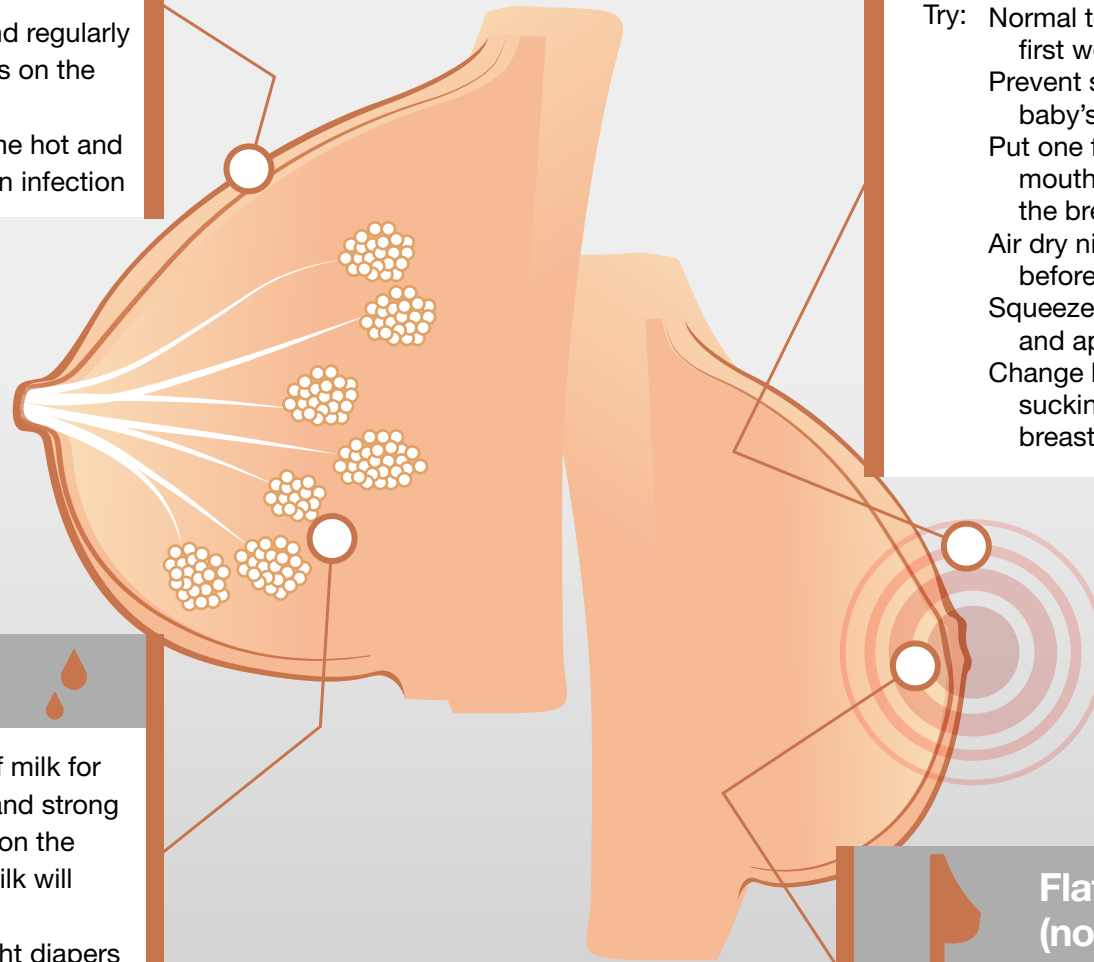


Try: Most mothers produce plenty of milk for their babies to grow healthy and strong
The more often the baby sucks on the mother's breasts, the more milk will come
A baby who is wetting six to eight diapers a day is getting plenty of milk
A baby's cry may be unrelated to hunger



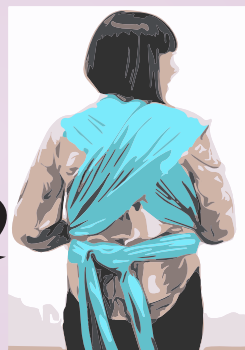
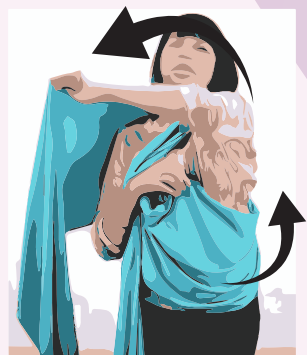
Flat or inverted nipples (not sticking out) ?

Try: Squeeze nipple then gently pull out nipple before putting it in baby's mouth



Kangaroo mother care

Newborn and new mother care | A4 poster | 1/1



Kangaroo mother care

A method of holding a baby that involves skin-to-skin contact. The baby, who is naked except for a diaper and a piece of cloth covering his or her back (either a receiving blanket or the parent's clothing), is placed in an upright position against a parent's bare chest.

Putting the baby in the wrap



Ways to keep newborns safe

Newborn and new mother care | A4 poster | 1/1



Signs that mother is having a problem after birth

Newborn and new mother care | A4 poster | 1/1

Fits



Shaking that does not stop

Swollen, red or very tender breasts



especially combined with a fever

Severe pain in the belly



different from the cramping some women feel when they first breastfeed the baby

Heavy bleeding



Soaks a pad or cloth in less than five minutes

Bleeding that smells bad



Pain in the chest or legs



Severe headache or trouble seeing



Vomiting



Fever and/or chills



Fast breathing or difficulty with breathing



Unable to pass urine or leaking urine



Too weak to get out of bed



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Post-abortion care

Post-abortion care | A4 poster | 1/2 - front

Take care to...

A Use clean cloths, pads or cotton wool for light bleeding and change every 2 to 4 hours.



B Drink plenty of water, tea, juice or other healthy fluids.



C Eat healthy foods, especially those rich in iron, such as meat, fish, eggs, beans, nuts and seeds, dried fruit and whole grains such as brown rice.



D Get plenty of rest.



E Avoid lifting anything heavier than four to five kilos or anything over your head for at least two to three days.



F Return to your normal life only when you feel better.



G Avoid sex or putting anything in the vagina for at least five days after the abortion and three days after the bleeding has stopped.



H Get some birth control as you can get pregnant again within ten days of the abortion.



I Wait at least six months to get pregnant so your body can recover.



Post-abortion care

Post-abortion care | A4 poster | 2/2 - back

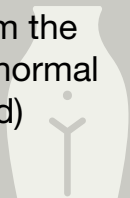
When should you visit the health worker?

Signs that there may be a problem after an abortion

Tell the woman about the signs that mean she must ask for help from a health worker if she sees any of them. They are:

A

Heavy bleeding from the vagina (more than a normal menstrual period)



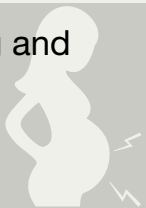
B

Bleeding that lasts more than two weeks after the abortion



C

Constant cramping and severe pain



D

Smelly liquid coming from the vagina



E

Vomiting or feeling sick



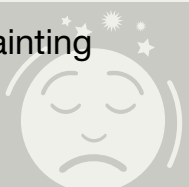
F

Fever or chills



G

Confusion or fainting



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Key messages about menstruation

Menstrual hygiene | A3 poster | 1/1

Women have a menstrual cycle which is usually around a month. This cycle helps their bodies prepare for pregnancy. It is a normal bodily function for women.



Menstruation is an indication that a girl is becoming a woman (in terms of being fertile) and usually starts between 10 to 15 years of age.



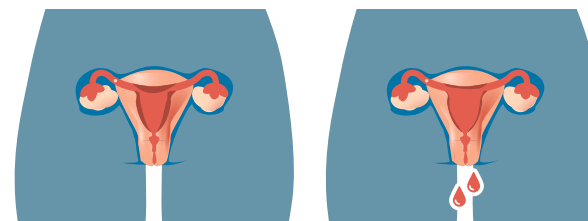
Some women feel cramps (pains in the lower stomach) just before and during menstruation, and especially on the first day. The cramps are normal. They are caused by contractions of the womb as it sheds the lining (the menstrual blood).



Bathing when you have your period is safe. Vaginas clean themselves on the inside. Only clean the outside of the vagina with gentle soap and water.



Having sex is safe during menstruation, but it depends on the culture whether it is taboo or not.



The period is the monthly flow of blood from the vagina when your body sheds the lining of the womb which is not needed if you are not pregnant.



Women usually stop having their periods between 45 to 55 years of age.



In normal circumstances, most women get their period every 28 days and the bleeding lasts for five to seven days, although the number of days between each period and the amount of bleeding varies from woman to woman, and especially when a girl is just beginning to get her period.



Some girls and women also experience pain or soreness in their breasts and back pain during menstruation.

Facts and misunderstandings about menstrual hygiene

Menstrual hygiene | A3 cut out sheet | 1/1



At least once a day during menstruation, wash the skin outside and around the vagina.	Do not wash inside the vagina (douche) because it can cause an infection.
Changes pads and tampons every hour.	Always use a clean and dry cloth or pad, a new tampon or a clean menstrual cup.
Do not share any sanitary protection with others.	Do not bathe during menstruation.
Change menstrual cloth or pad (both disposable and reusable) and tampons three to six times a day, depending on the flow.	At least once a day during menstruation, wash inside the vagina (douche).
You can use a menstrual cup for up to 24 hours before cleaning.	When using a menstrual cup empty and wash every four to eight hours.
Using dirty or damp cloths and pads can lead to itching or infection.	Wash hands after changing your sanitary protection to remove any blood or germs on your hands.
Wash reusable menstrual cloths or pads with soap and water and, if possible, dry in the sun.	Rinse out menstrual cloths or pads with water only.
Douching or semen can clean the vagina.	

MISUNDERSTANDING

FACT



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Birth control methods chart

Family planning | A3 poster | 1/2 - front

METHOD		HOW IT PREVENTS PREGNANCY	HOW IT IS USED	OTHER INFORMATION	EFFECTS
Abstinence		By not having sex, semen is unable to cause pregnancy.	The man and woman do not have vaginal sex.	While it is very effective, abstinence can be difficult to practise as it does not allow for any sex.	
Breastfeeding		May prevent ovulation for new mothers who breastfeed exclusively day and night if their monthly bleeding has not returned.	Can be used until infant is six months old.	Breastfeeding may help to delay fertility, available without prescription. It is not 100% effective UNLESS the baby is being breastfed exclusively. If giving other food or if baby is older than six months, mother should use other methods.	
Fertility awareness methods (FAM)		Prevents pregnancy by avoiding unprotected vaginal sex during most fertile days of woman's cycle.	Woman track their fertile periods (usually days 8–19 of each 26 to 32-day cycle) using beads, checking consistency of mucus inside the cervix, taking body temperature at the same time every morning before getting out of bed or other aids.	FAM has a high failure rate. It is difficult to predict ovulation or fertile time very accurately. It requires correct and consistent use. Partner must agree as there must be some days when the couple cannot have vaginal sex. May be difficult to use if woman has a vaginal infection or other condition that changes cervical mucus or body temperature.	
Withdrawal method		By withdrawing the penis from the vagina before spilling semen, sperm cannot reach the egg in the woman's body.	The man withdraws his penis from the vagina without spilling any semen.	Not overly effective, does not protect against sexually-transmitted infections including HIV/AIDS.	
Birth control implant		Small flexible rod or rods placed under the skin which release hormones that prevent ovulation.	Once inserted no need to do anything and prevents pregnancy for three to five years.	Health care professional must insert and remove, irregular vaginal bleeding common but not harmful, can be inserted six weeks after childbirth while breastfeeding. Sometimes this is not tolerated by certain people's bodies.	
Birth control pill		Prevents the release of eggs from the woman's ovaries (stops ovulation).	Need to take one pill every day without fail. If you miss more than two pills in a cycle, use another method.	Only available from doctor or clinic, can start taking six months after childbirth if breastfeeding, reduces cramps due to menstruation, reduces risk of some cancers that women can get but may increase risk of stroke.	
Birth control shot		Hormones prevent ovulation by blocking release of egg.	One shot/injection every month or three months.	Available at clinic or from doctor, may be used while breastfeeding six weeks after childbirth, irregular vaginal bleeding or no menstruation common but not harmful.	
Diaphragm		It is a shallow, bendable cup that fits inside the vagina.	The diaphragm covers the cervix and does not allow sperm inside the uterus.	Prescription is required, diaphragm is reusable, should be used with a spermicide, will not protect from STDs.	
Emergency contraceptive pill		Pills taken to prevent pregnancy up to five days after having vaginal sex without using a method of contraception. In some instances a copper IUD may also be inserted which will protect you for an additional 10 years.	To be used only on occasion if unprotected sex has occurred. Should not be used as a regular birth control method. Do not use more than three times in one year.	In some countries, available without prescription. If already pregnant more than five days, it will not cause an abortion.	
Female condom		Sheath or lining made of thin, soft plastic that fits loosely inside a woman's vagina to prevent sperm and egg from meeting.	Used every time couple have vaginal, oral or anal sex.	Available without prescription, protects against sexually-transmitted infections including HIV/AIDS, can make sounds during sex, may be used while breastfeeding.	



**Most Effective
at Preventing
Pregnancy**



**Easiest to Use
When Having
Sex**



Prevents STDs



**Makes periods
lighter and less
painful**



**Requires visit
to health
facility/nurse
or doctor**



**Unreliable
method**



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Birth control methods chart

Family planning | A3 poster | 2/2 - back

METHOD		HOW IT PREVENTS PREGNANCY	HOW IT IS USED	OTHER INFORMATION	EFFECTS
Female sterilization (tubal ligation)		Fallopian tubes (the tubes that carry the eggs) blocked or cut to prevent egg from meeting sperm.	Once performed by health professional requires nothing further to prevent pregnancy.	Woman must be given good information and do this voluntarily. If not done within seven days of childbirth must wait at least six weeks to do procedure.	
IUD (intrauterine device)		Small device inserted into the uterus prevents the egg becoming implanted in women's uterus.	Once inserted by a health provider no need to do anything and prevents pregnancy for up to 10 years.	Health care professional must insert and remove. Can be used while breastfeeding. If not inserted within 48 hours of childbirth, must wait for four to six weeks to insert.	
Male condom		Sheath or covering that fits over a man's erect penis and prevents sperm from entering the vagina.	Requires use every time couple have vaginal sex.	Available without prescription, protects against sexually-transmitted infections including HIV/AIDS. Expiry dates should be checked before use. May be used while breastfeeding.	
Male sterilization (vasectomy)		Tubes that carry sperm from the testicles are blocked or cut.	Once procedure is performed by health professional, there is a three-month delay in taking effect while stored sperm is present.	Does not affect male sexual performance. The man must be given good information and do this voluntarily.	
Vaginal ring		A small, flexible ring placed up in the vagina stops the semen from reaching the egg in the woman's body by ensuring there are no eggs to fertilise. Hormones also makes it hard for sperm to "swim" in the woman's body.	By pushing the sides of the ring together and pushing up in the vagina, the woman is protected from pregnancy. After one month, it is easily removeable and replaced with another ring.	Requires visit to clinic or health worker. Needs to be replaced every month. Rings can expire.	
Traditional methods		Many traditional methods are shared based on culture. They are not scientifically reliable in preventing pregnancy and some can hurt the woman.	Many methods exist around the world and can be based on religious beliefs, customs, cultures, etc. The method varies by the geographic region.	While eCBHFA volunteers may personally believe in these methods, the Red Cross Red Crescent does not promote traditional methods as they are not scientifically based and may cause harm.	



Which method is best for me?

Family planning | A4 form | 1/1

	 Most effective at preventing pregnancy	 Easiest to use when having sex	 Prevents STDs	 Makes periods lighter and less painful	 Requires visit to health facility/nurse or doctor	 Unreliable method
Birth control implant	✓	✓		✓	✓	
IUD	✓	✓		✓	✓	
Birth control shot	✓	✓		✓	✓	
Birth control vaginal ring	✓	✓		✓	✓	
Birth control patch	✓	✓		✓	✓	
Birth control pill	✓	✓		✓	✓	
Diaphragm					✓	
Emergency contraception					✓	
Cervical cap					✓	
Male condom			✓			
Female condom			✓			
Male vasectomy	✓	✓			✓	
Female sterilization	✓	✓			✓	
Abstinence						✓
Withdrawal						✓
Breastfeeding						✓
Fertility awareness methods						✓
Traditional methods						✓

Using male condoms

Family planning | A3 poster | 1/1

STEP 1



21 JAN 2020

Check expiry date on condom package.

STEP 2



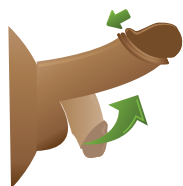
Tear open condom package with your hands being careful not to rip the condom.

STEP 3



Check that the tip of the condom is pushed through the outside of the coiled ring.

STEP 4



Make sure the penis is erect and pull back the foreskin if necessary

STEP 5



Pinch the tip of the condom and place the condom over the head of the erect penis.

STEP 6



Unroll condom to the base of the erect penis.

STEP 7



Smooth out air bubbles before inserting the penis into the vagina or anus.

STEP 8



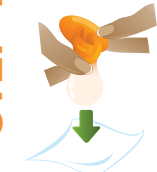
After sex, carefully withdraw penis before penis becomes soft – carefully holding condom to ensure no semen is spilled.

STEP 9



Pinch condom at the tip to keep semen inside the condom and slide condom off penis.

STEP 10



Tie the condom in a knot and collect in a tissue or other disposable material.

STEP 11



Throw away used condom in a garbage bin.

STEP 12



Wash your hands.

Using male condoms

Family planning | A4 cut & paste activity | 1/1



Using female condoms

Family planning | A3 poster | 1/1

STEP 1



21 JAN 2020

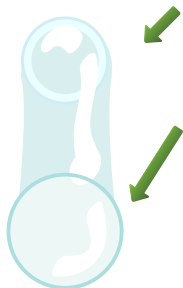
Check expiry date on condom package.

STEP 2



Tear open condom package with your hands being careful not to rip the condom.

STEP 3



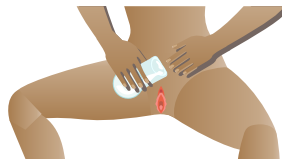
The inner ring of the condom goes inside the vagina where it helps to hold the sheath (condom) in place during the vaginal sex. The outer ring of the condom covers the area around the opening of the vagina.

STEP 4



While holding the condom at the closed end, grasp the flexible inner ring with the thumb and forefinger and squeeze it so it becomes long and narrow to make it easier to insert into the vagina.

STEP 5



Choose a position to insert the condom that is comfortable such as squatting, lying down or with one leg raised with your foot on a chair or stool or on the foot of the bed.

STEP 6



Gently insert the inner ring into the vagina, feeling it go up and move into place. Place your index finger inside the condom and push the inner ring as far up as it will go. Be sure the condom is not twisted and the outer ring remains on the outside of the vagina.

STEP 7



The condom is now in place and ready to use with your partner. Gently guide the erect penis into the opening of the condom with your hand to make sure the penis enters the inside of the condom.

STEP 8



After the male orgasm remove the condom by twisting the outer ring and gently pull the condom out.

STEP 9



Wrap the condom in the package or paper and throw in a rubbish bin (not in a toilet or latrine).

STEP 10

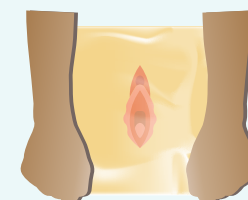
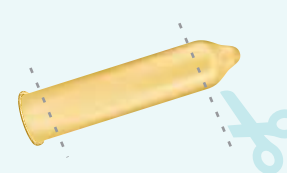


Wash your hands.



Having sex and no penis involved? Use a dental dam.

Cover the vagina to protect from STDs during sex.

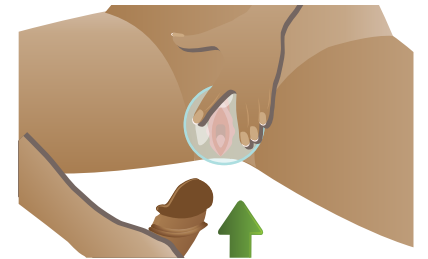
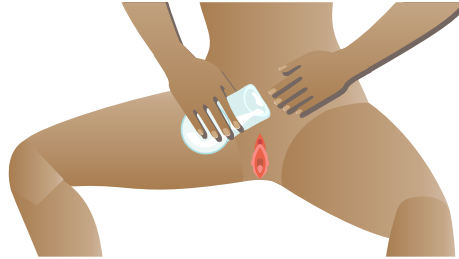


Using female condoms

Family planning | A4 cut & paste activity | 1/1



21 JAN 2020



Reasons why female cutting (FGM/C) still exists

Female genital mutilation/cutting | A4 poster | 1/1

Some people may believe that...

...it makes a woman pure.

...it is a rite and a necessary part of a community's culture and identity.

...it is a mark of good hygiene and beauty.

...it is required as part of their religion.

Some people...

...make their money from cutting and so will promote cutting as a way to make money using the reasons to the left.



But these are not worth the risks



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What religion says about female cutting

Female genital mutilation/cutting | A4 poster | 1/1



**of the major religion's texts support or teach
female circumcision or cutting.**

Effects of female cutting on women's health

Female genital mutilation/cutting | A4 poster | 1/1

Immediate problems

Severe pain



Excessive bleeding



Genital tissue swelling



High fever



Infections like tetanus or HIV



Constantly leaking urine



Cut areas not healing



Shock



Death



Long-term problems

 Infertility / loss of baby-bearing

 Risk of childbirth problems

 Death of the newborn

 Urinary problems

 Vaginal problems

 Menstrual problems

 Scar tissue and long-term pain

 Sexual problems and pain

 Need for later surgery

 Depression / anxiety / PTSD



Cutting is a violation of a child's rights when performed on a child below the age of consent